



PROCEDURE

Management of Potential Cervical Spine Injuries in Under 16 Year Olds

TABLE OF CONTENTS

TARGET AUDIENCE and SETTING.....	1
PURPOSE	1
PRECAUTIONS/CONTRAINDICATIONS	1
EQUIPMENT.....	1
STANDARD REQUIREMENTS.....	2
PROCEDURE.....	2
RELATED DOCUMENTATION	4
KEYWORDS	4

TARGET AUDIENCE and SETTING

This procedure is applicable to all Monash Health staff, patients, clients and their families and carers when the patient is less than 16 years old.

Note students work under the direct supervision of employees and are not listed separately.

PURPOSE

Provide clear guidance for the management of potential cervical spine injuries in children in Monash Health Emergency Departments.

PRECAUTIONS/CONTRAINDICATIONS

Discuss all cases with senior ED clinician and/ or neurosurgical registrar

EQUIPMENT

- RCH (Victorian) guideline for cervical spine assessment:
http://www.rch.org.au/clinicalguide/guideline_index/Cervical_Spine_Assessment/
- Paediatric radiologist via Monash Health switchboard 9594 6666 (ext. 92) between 0900 and 1700
- Radiology on call registrar available via Monash Health switchboard 9594 6666 (ext. 92) between 1700 and 0900
- Paediatric Radiology Consultant 8572 3226 (ext. 23226) between 0900 and 1700
- Neurosurgical registrar available via Monash Health switchboard 9594 6666 (ext. 92)

- Monash Medical Centre Paediatric Emergency Senior Doctor 8572 5145 (ext. 25145)
- Dandenong Emergency Senior Doctor 9554 9960 (ext. 49960)
- Casey Emergency Senior Doctor 8768 1849 (ext. 81849)
- Monash Health Bed Manager via Baret. Out of hours contact nursing coordinator.

STANDARD REQUIREMENTS

When undertaking any clinical interaction with a patient, staff are expected to:

- Perform routine hand hygiene. Refer to the [Hand Hygiene Procedure](#).
- Introduce yourself to the Patient and Carer/ Family if in attendance.
- Explain what you do and what you are there for
- Check patient identification. Refer to the [Patient Identification Procedure](#).
- Obtain consent as per the [Consent to Medical Treatment Procedure](#).
- Keep the patient/carer informed and involve them in decision making.
- Document interaction (such as patient review, assessments, care plans and procedures) in the appropriate location within the electronic medical record (EMR) **or** paper health record using black or blue pen; including date, time, signature and designation. Refer to [Clinical Documentation in the Health Record](#)

PROCEDURE

Monash Health endorses the Safer Care Victoria Paediatric Improvement Collaborative (PIC) clinical practice guideline on cervical spine assessment. The document can be found here:

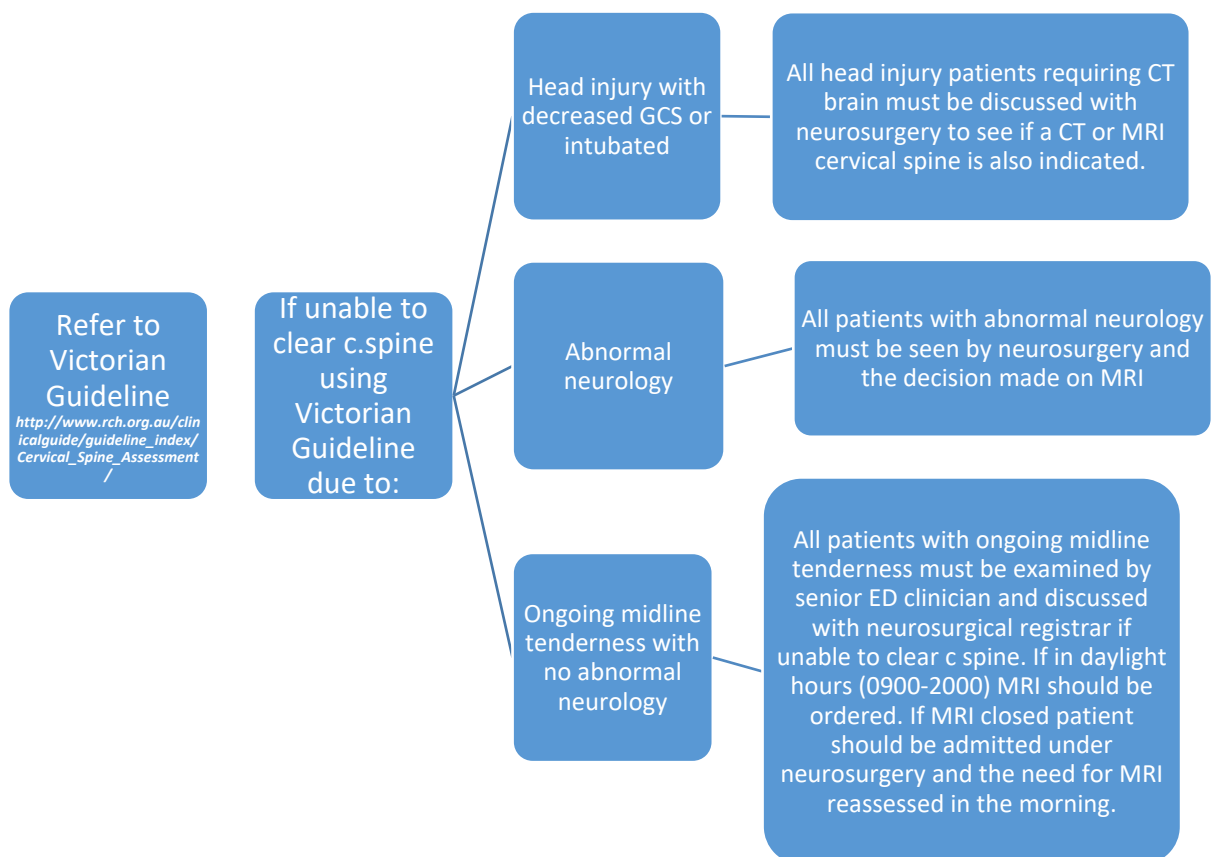
https://www.rch.org.au/clinicalguide/guideline_index/Cervical_spine_assessment/

1. Use this PROMPT guideline **only** if unable to clear the cervical spine using the PIC guideline.
2. Prescribe regular analgesia
3. After discussion with the emergency department (ED) doctor in charge, refer all patients under 16 years old with abnormal neurology with potential cervical spine injury to neurosurgical registrar who must review the patient and decide whether an MRI is needed.
 - a. In general, CT must not be performed because it cannot exclude an unstable injury and has a moderate dose of radiation.
4. All patients with ongoing midline tenderness with normal neurology must be examined by the senior ED clinician. If the senior ED clinician is unable to clear cervical spine, then refer to neurosurgical registrar.
5. Request an MRI. Between 0900 and 1700 this would be a discussion between the senior ED clinician and Paediatric Radiologist (tel: 85723226). If between 1700 and 2000, the discussion needs to be with the on-call MRI consultant who is available via switch. When requested between 0900 and 2000 radiology will endeavour to do an MRI that day, if not

possible the patient must be admitted under neurosurgery and reassessed in the morning. Patients must remain in a foam collar as per PIC guideline:

http://www.rch.org.au/clinicalguide/guideline_index/Cervical_Spine_Assessment/

- Ensure the collar and label "C-spine not cleared" remain in place until C - Spine is cleared for patients not meeting exceptions as per guideline.
- 6. Out of hours (2000 to 0900), if neurosurgical intervention is required, then neurosurgical registrar (after seeing patient) must assess whether out of hours MRI is required. Out of hours MRI is not required for ongoing midline tenderness without any significant neurology.
- 7. Patients under the age of 16 years old in Casey or Dandenong EDs whose cervical spines cannot be cleared using PIC guideline for cervical spine assessment and after senior ED clinician review must be discussed with neurosurgical registrar
 - a) If an MRI is deemed necessary and is available at referring site it must be performed at referral site.
 - b) If the patient needs transfer to Monash Children's Hospital (MCH) and there is a bed at MCH, the patient is admitted directly to ward. If there is no bed available on the ward the patient would be transferred to MMC ED.
 - c) If referring a patient out of hours and neurosurgical registrar deems urgent review not necessary, then the patient must remain at referral site in a foam collar and receive regular analgesia and have senior ED clinician review in the morning. If cervical spine still unable to be cleared, the patient needs neurosurgical review at MMC.



RELATED DOCUMENTATION

Monash Health wish to acknowledge the use of PIC Guideline for Cervical Spine Assessment in the preparation of this procedure:

[https://www.rch.org.au/clinicalguide/guideline_index/Cervical_spine_assessment/
Clinical Observations and Assessment \(Paediatric\)](https://www.rch.org.au/clinicalguide/guideline_index/Cervical_spine_assessment/Clinical_Observations_and_Assessment_(Paediatric))

KEYWORDS

Cervical spine; MRI; children; paediatric; collar

Document governance	
Policy or Framework supported by this Procedure	High Quality Care (Policy)
Executive Sponsor	Operations Director, Children's Program
Responsible Program, Service, Unit, Department or Committee	Children's Program; Paediatric Emergency Department
Primary Contact	Director of Paediatric Emergency Medicine

PROMPT Doc No: SNH0178971 v4.0		
Date loaded on PROMPT: 15/10/2021	Page 4 of 4	Review By: 28/02/2030
Version Changed: 09/02/2026	Document uncontrolled when downloaded.	Last Reviewed Date: 09/02/2026